

Daily Content Intel

August 17, 2026

Topic A — Reel

TELEPROMPTER COPY (paste into Instagram)

In a study of 539 teenage ACL athletes, the ones who tore their other knee were the most confident in the room.

Okay, here's the setup. Every one of them filled out the ACL return to sport questionnaire before surgery and again at 3, 6, and 9 months, and the researchers followed them for 5 years. 22.2% tore an ACL again. Roughly half of those were the surgical knee and half were the opposite one.

Now the part that flips it. The male athletes who tore the opposite ACL had scored 85 on that confidence questionnaire at 9 months, while the uninjured group sat at 77. The female

athletes who tore the opposite ACL were already sitting high at 6 months. The male athletes who blew out the graft again went the other direction, down at 66.

So the same score points two ways, and which way depends on the knee you're asking about. A low score flags the surgical leg. A score that climbs fast and parks high flags the leg that isn't getting tested.

Here's what I'd do Monday. Score it more than once, watch the slope instead of the number, and put the healthy leg through the same hop and strength testing as the reconstructed one.

Be Good.

Help Someone.

Learn Lots.

Hook: In a study of 539 teenage ACL athletes, the ones who tore their other knee were the most confident in the room.

Spoken script (word for word):

In a study of 539 teenage ACL athletes, the ones who tore their other knee were the most confident in the room.

Okay, here's the setup. Every one of them filled out the ACL return to sport questionnaire before surgery and again at 3, 6, and 9 months, and the researchers followed them for 5 years. 22.2% tore an ACL again. Roughly half of those were the surgical knee and half were the opposite one.

Now the part that flips it. The male athletes who tore the opposite ACL had scored 85 on that confidence questionnaire at 9 months, while the uninjured group sat at 77. The female athletes who tore the opposite ACL were already sitting high at 6 months. The male athletes who blew out the graft again went the other direction, down at 66.

So the same score points two ways, and which way depends on the knee you're asking about. A low score flags the surgical leg. A score that climbs fast and parks high flags the leg that isn't getting tested.

Here's what I'd do Monday. Score it more than once, watch the slope instead of the number, and put the healthy leg through the same hop and strength testing as the reconstructed one.

Be Good.

Help Someone.

Learn Lots.

On-screen text seeds:

- 0:00 539 teen ACL athletes. 5 years.
- 0:08 22.2% tore an ACL again
- 0:20 Opposite knee: 85 vs 77
- 0:32 Same knee, graft failure: 66 vs 77
- 0:45 Low score = surgical leg at risk
- 0:52 High, fast-climbing score = other leg at risk
- 1:05 Test the healthy leg the same way

Caption (copy-paste ready):

539 adolescents were tracked for 5 years after ACL reconstruction. 22.2% tore an ACL again, and it split almost evenly between the surgical knee and the other one.

The confidence questionnaire told two different stories. Male athletes who went on to tear the opposite ACL had scored 85 at 9 months against 77 for the uninjured group. Male athletes who failed the graft scored 66. A single cutoff would have missed both.

That's why I score readiness on a curve instead of a checkpoint, and why the uninjured leg gets tested like it's the injured one.

Being ready to play and being safe to play are two different measurements.

Book at threshold.clientsecure.me

Dr. Lars Stevenson, PT, DPT

#ACLrehab #returntosport #lacrosse

#footballtraining #sportsPT

#athletedevelopment #kneerehab

#strengthandconditioning #physicaltherapy

#youthsports

Personalize this

- Name a return-to-sport call you made where the athlete's confidence was running ahead of their testing. What did you see on the hop or change-of-direction work that the athlete didn't feel?
- Where does the uninjured limb sit inside CROSS for you? Walk through how you actually test the healthy leg on an ACL athlete at 9 months, and what number would make you hold them.
- Football and lacrosse both put a huge cutting load on the plant leg. Do you see the opposite-knee risk show up differently between your football guys and your club lacrosse athletes?

Shoot notes:

- Say "539" as the very first sound out of your mouth with the camera already rolling. The brief's median 27.5pp drop happens in the first 8 seconds on reels that open mid-sentence, so start the take on the number, not on a breath.

- Shoot this one against a flat clinic wall, not the open gym. Two of the three logged drop-off frames had a visually busy gym background or large windows pulling attention off you.
- Frame chest-up with your head near the top third of the frame. One flagged drop-off frame was "plain white ceiling dominating the upper half" and another was "large empty wall space dominating the upper frame." Kill the dead space above your head.
- Energy up on the 85 versus 77 beat. The flagged frames all read "flat expression, low energy" at the moment viewers left.

Hook shape: Story hook — "Story hook: 60.9% (-3.2pp vs average, n=4)" is the only opener shape below Lars's all-reel skip average, and the claim (the most confident athletes were the ones who got hurt) lands inside the first line rather than after a wind-up.

Screenshots:

- title | <https://journals.sagepub.com/doi/10.1177/23259671251410107> |

"Postoperative Anterior Cruciate Ligament–Return to Sport after Injury Scores Predict Reinjury in Adolescent Athletes"

- physiology | <https://journals.sagepub.com/doi/10.1177/23259671251410107> | "The rate of second ACL injury was 22.2% (ipsilateral 9.7%; contralateral 12.5%) at 5 years"
- actionable | <https://journals.sagepub.com/doi/10.1177/23259671251410107> | "Low scores during rehabilitation identify athletes vulnerable to ipsilateral graft failure, whereas rapidly rising, high scores indicate those at risk for contralateral reinjury"

Sources:

- <https://journals.sagepub.com/doi/10.1177/23259671251410107>

Topic B — Reel

TELEPROMPTER COPY (paste into Instagram)

The bottom half of the rep is doing almost all of the muscle building work, and it's the half that gets skipped in most high school weight rooms.

Okay, here's where that comes from. Researchers took 30 trained lifters, gave one arm a full range of motion and gave the other arm only the stretched bottom half, roughly 50% of that person's range, and ran it twice a week for 8 weeks. Muscle thickness came out the same. Strength endurance came out the same. The statistics actually leaned toward no difference at all.

The meta-analysis on this says the same thing. A trivial edge for full range, 0.12, which in a weight room is nothing, and the group training partials at long muscle lengths may have grown a little more.

So the cue I'd change is depth. Police how low the bar gets on every rep, on every set, and treat the loaded stretched position as the whole point of the lift. The quarter squat at the top of the bar path is the one buying you nothing.

I'll hedge the evidence, because it's honest: trained adults, upper body, 8 weeks, small sample. I won't hedge the coaching. Full range stays the default in my programs because it's easier to coach and it protects range. Just quit letting anyone cut the bottom.

Be Good.

Help Someone.

Learn Lots.

Hook: The bottom half of the rep is doing almost all of the muscle building work, and it's the half that gets skipped in most high school weight rooms.

Spoken script (word for word):

The bottom half of the rep is doing almost all of the muscle building work, and it's the half that gets skipped in most high school weight rooms.

Okay, here's where that comes from.

Researchers took 30 trained lifters, gave one arm a full range of motion and gave the other arm only the stretched bottom half, roughly 50% of that person's range, and ran it twice a week for 8 weeks. Muscle thickness came out the same. Strength endurance came out the same. The statistics actually leaned toward no difference at all.

The meta-analysis on this says the same thing. A trivial edge for full range, 0.12, which in a weight room is nothing, and the group training partials at long muscle lengths may have grown a little more.

So the cue I'd change is depth. Police how low the bar gets on every rep, on every set, and treat the loaded stretched position as the whole point of the lift. The quarter squat at the top of the bar path is the one buying you nothing.

I'll hedge the evidence, because it's honest: trained adults, upper body, 8 weeks, small sample. I won't hedge the coaching. Full range stays the default in my programs because it's easier to coach and it protects range. Just quit letting anyone cut the bottom.

Be Good.

Help Someone.

Learn Lots.

On-screen text seeds:

- 0:00 The bottom half is the whole lift
- 0:10 30 lifters. 8 weeks. One arm full ROM, one arm bottom half only.
- 0:24 Same muscle thickness. Same strength endurance.
- 0:36 Meta-analysis edge for full ROM: 0.12
- 0:48 Coach depth, not rep count
- 1:00 Trained adults, upper body, 8 weeks
- 1:10 Full range still the default. Stop cutting the bottom.

Caption (copy-paste ready):

30 trained lifters. One arm trained a full range of motion, the other arm trained only the stretched bottom half, about 50% of the range. 8 weeks later the muscle thickness and the strength endurance came out the same.

The meta-analysis backs it up. A 0.12 edge for full range, which is nothing you'd ever feel, and partials at long muscle lengths may actually have an advantage for size.

That changes what I police in the weight room. Depth, every rep, every set. The stretched position under load is where the adaptation lives, and the quarter squat at the top of the bar path is the part earning nothing.

Fair caveats: trained adults, upper body, 8 weeks, small sample. Full range stays my default because it's easier to coach and it keeps range. The bottom is what I refuse to give up.

Book at threshold.clientsecure.me

Dr. Lars Stevenson, PT, DPT

#strengthandconditioning #footballtraining
#lacrosse #weightroom #hypertrophy
#athletedevelopment #highschoolsports
#squat #rangeofmotion #sportsPT

Personalize this

- What does depth policing look like in the football programs you write? Give a concrete example of a lift where you changed the setup (box, pins, handles, speed of movement) to force the athlete into the stretched position.
- You've had athletes come to you strong on paper and weak in the bottom of a squat or split squat. Tell one of those stories, and what the loaded stretch exposed that their numbers hid.
- Where does this fit for a post-op or late-stage rehab athlete who can't access full range yet? That's the exact case the meta-analysis calls out, and it's your lane.

Shoot notes:

- Cold open with "The bottom half of the rep" already in motion. No breath, no "so", no wind-up. The brief's single biggest leak

is a mid-sentence, delayed-payoff opener in the 0 to 8 second window.

- This one wants the weight room, but stand so the rack or a single plate tree is behind you and the rest is empty. The logged drop-off frame was "gym equipment competing for attention" with "large windows" behind the subject, so pick the quietest wall in the room.
- Get closer and drop the camera to chest height so your head sits high in the frame. Two flagged frames lost viewers to empty space above the head.
- This is the opinion reel, so bring the energy that the flagged frames were missing, especially on "the quarter squat at the top is buying you nothing." Say that line looking straight down the lens.

Hook shape: Statement hook — the brief permits a statement opener "only when its first line carries the claim itself rather than a setup for one," and this line states the finding and names the coaching failure in one breath. Story shape was skipped here because it would have pushed the claim to

the second sentence, which is exactly the 0 to 8 second failure the brief flags.

Screenshots:

- title | <https://pmc.ncbi.nlm.nih.gov/articles/PMC11829627/> | "Lengthened partial repetitions elicit similar muscular adaptations as full range of motion repetitions during resistance training in trained individuals"
- physiology | <https://pmc.ncbi.nlm.nih.gov/articles/PMC11829627/> | "Both muscle thickness and unilateral lat pulldown 10-repetition-maximum improvements were similar between the two conditions"
- actionable | <https://pmc.ncbi.nlm.nih.gov/articles/PMC11829627/> | "Trainees seeking to maximize muscle size should likely emphasize the stretched position, either by using a full ROM or LPs during upper-body resistance training"

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC11829627/>

- <https://journal.iusca.org/index.php/Journal/article/view/182>

Reference

Runner-up topics

- Low energy availability and in-season injury in adolescent athletes, paired with the finding that adolescent athletes average 6.3 hours of sleep a night. Strong fueling-and-sleep teaching reel for football two-a-days.
- BPC-157 human evidence versus the marketing, including WADA S0 status and the ~30% mislabeling rate in grey-market peptide samples. Blocked today by the 90-day source dedup; re-run after 2026-09-20 with a fresh primary source (the 2025 narrative review, PMC12446177).
- IronRadio Ep 248's other thread: 11-year-old national squat records and what "rapid strength progress in young lifters" actually means for youth resistance training dosing.

Sources scanned today

- **Newsletter digest 2026-08-17** (~/Cowork /newsletter-digest/output/newsletter_diges

t_2026-08-17.pdf, 3 sources). Dr. Mercola x2 (mercury fillings, diabetes) = pure promo, skipped. Mike T Nelson Weekly Wrap Up Aug 16 = the real signal: his BPC-157 Reality Check PDF ("35 rat studies... human evidence almost zero"), Flex Diet Ep 399 with Dr. Emily Werner on urolithin A / mitochondrial health, and IronRadio Ep 248 (Aug 10) on youth powerlifting records plus Lonnie's "quarter-rep problem" debate about partial-ROM reps on machines. The quarter-rep thread became Draft B.

- **PubMed / OJSM / AJSM (ACL, adolescent, return-to-sport)** — surfaced two 2026 papers. McAleese et al., *Orthopaedic Journal of Sports Medicine*, May 2026, ACL-RSI serial scores in 539 adolescents: read in full, became Draft A. Maheshwer et al., *AJSM* 2026 (431 adolescents, younger age and earlier RTS predict second injury): paywalled, could not read the full text, so it was dropped rather than cited unread.
- **Partial vs full range of motion literature** — Wolf et al., *PeerJ* 2025 (lengthened

partials, n=30, 8 weeks) and Wolf et al., *IJSC* 2023 systematic review and meta-analysis. Both read in full. Became Draft B.

- **Low energy availability / adolescent sleep** — scanned (LEA and in-season injury in female adolescent athletes, adolescent sleep averaging 6.3 h/night). Strong material, held as a runner-up.
- **BPC-157 / WADA peptide angle:** genuinely the loudest item in the digest and the obvious hot take, but **rejected at the Step 3.5 dedup gate:** both usada.org/spirit-of-sport/bpc-157-peptide-prohibited/ and the StatNews BPC-157 piece were cited on 2026-06-22, inside the 90-day window. Swapped to the runner-up (quarter reps).
- **Unreachable:** pubmed.ncbi.nlm.nih.gov direct fetch (cookie wall), journals.sagepub.com/doi/10.1177/03635465251395297 (AJSM paywall, returned nav shell only).